# ACT Onco® + Report

| PATIENT | | |
|---|---|---|
| Identifier: 陳政賢 | | Patient ID: 48398465 |
| Date of Birth: Apr 21, 1973 | | Gender: Male |
| Diagnosis: Epithelioid tumor | | |
| **ORDERING PHYSICIAN** | | |
| Name: 顏厥全醫師 | | Tel: 886-228712121 |
| Facility: 臺北榮總 | | |
| Address: 臺北市北投區石牌路二段 201 號 | | |
| **SPECIMEN** | | |
| Specimen ID: S11218466A | Collection site: Soft tissue, forearm | Type: FFPE tissue |
| Date received: May 25, 2023 | Lab ID: AA-23-03415 | D/ID: NA |

**ABOUT ACTOnco®+**

The test is a next-generation sequencing (NGS)-based assay developed for efficient and comprehensive genomic profiling of cancers. This test interrogates coding regions of 440 genes associated with cancer treatment, prognosis and diagnosis. Genetic mutations detected by this test include small-scale mutations like single nucleotide variants (SNVs), small insertions and deletions (InDels) (≤ 15 nucleotides) and large-scale genomic alterations like copy number alterations (CNAs). The test also includes an RNA test, detecting fusion transcripts of 13 genes.

## SUMMARY FOR ACTIONABLE VARIANTS

### VARIANTS/BIOMARKERS WITH EVIDENCE OF CLINICAL SIGNIFICANCE

| Genomic Alterations/Biomarkers | Probable Effects in Patient's Cancer Type | | Probable Sensitive in Other Cancer Types |
|---|---|---|---|
| | Sensitive | Resistant | |
| Not detected | | | |

### VARIANTS/BIOMARKERS WITH POTENTIAL CLINICAL SIGNIFICANCE

| Genomic Alterations/Biomarkers | Possibly Sensitive | Possibly Resistant |
|---|---|---|
| Not detected | | |

**Note:**

- The above summary tables present genomic variants and biomarkers based on the three-tiered approach proposed by US FDA for reporting tumor profiling NGS testing. "Variants/biomarkers with evidence of clinical significance" refers to mutations that are widely recognized as standard-of-care biomarkers (FDA level 2/AMP tier 1). "Variants/biomarkers with potential clinical significance" refers to mutations that are not included in the standard of care but are informational for clinicians, which are commonly biomarkers used as inclusion criterial for clinical trials (FDA level 3/AMP tier 2).
- The therapeutic agents and possible effects to a given drug are based on mapping the variants/biomarkers with ACT Genomics clinical knowledge database. The mapping results only provide information for reference, but not medical recommendation.
- Please refer to corresponding sections for more detailed information about genomic alteration and clinical relevance listed above.

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## TESTING RESULTS

### VARIANT(S) WITH CLINICAL RELEVANCE

#### - Single Nucleotide and Small InDel Variants

| Gene | Amino Acid Change | Allele Frequency |
|---|---|---|
| Not detected | | |

#### - Copy Number Alterations

| Chromosome | Gene | Variation | Copy Number |
|---|---|---|---|
| Chr22 | *CHEK2, NF2* | Heterozygous deletion | 1 |

#### - Fusions

| Fusion Gene & Exon | Transcript ID |
|---|---|
| No fusion gene detected in this sample | |

#### - Immune Checkpoint Inhibitor (ICI) Related Biomarkers

| Biomarker | Results |
|---|---|
| Tumor Mutational Burden (TMB) | 0.0 muts/Mb |
| Microsatellite Instability (MSI) | Microsatellite stable (MSS) |

Note:
- Loss of heterozygosity (LOH) information was used to infer tumor cellularity. Copy number alteration in the tumor was determined based on 33% tumor purity.
- For more therapeutic agents which are possibly respond to heterozygous deletion of genes listed above, please refer to APPENDIX for more information.
- TMB was calculated by using the sequenced regions of ACTOnco®+ to estimate the number of somatic nonsynonymous mutations per megabase of all protein-coding genes (whole exome). The threshold for high mutation load is set at ≥ 7.5 mutations per megabase. TMB, microsatellite status and gene copy number deletion cannot be determined if calculated tumor purity is < 30%.
- SMARCB1 synonymous mutation (c.1116G>A, p.Thr372=) and SMARCB1 heterozygous deletion (copy number = 1) were detected in the specimen.

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# THERAPEUTIC IMPLICATIONS

## TARGETED THERAPIES

Not Applicable.

## IMMUNE CHECKPOINT INHIBITORS (ICIs)

No genomic alterations detected to confer sensitivity or lack of benefit to immune checkpoint therapies.

## - Other Biomarkers with Potential Clinical Effects for ICIs

| Genomic Alterations | Potential Clinical Effects |
|---|---|
| Not detected | |

Note: Tumor non-genomic factors, such as patient germline genetics, PDL1 expression, tumor microenvironment, epigenetic alterations or other factors not provided by this test may affect ICI response.

## CHEMOTHERAPIES

No genomic alterations detected in this tumor predicted to confer sensitivity or lack of benefit to chemotherapies.

## HORMONAL THERAPIES

No genomic alterations detected in this tumor predicted to confer sensitivity or lack of benefit to hormonal therapies.

## OTHERS

No genomic alterations detected in this tumor predicted to confer sensitivity or lack of benefit to other therapies.

**Note:**
Therapeutic implications provided in the test are based solely on the panel of 440 genes sequenced. Therefore, alterations in genes not covered in this panel, epigenetic and post-transcriptional and post-translational factors may also determine a patient's response to therapies. In addition, several other patient-associated clinical factors, including but not limited to, prior lines of therapies received, dosage and combinations with other therapeutic agents, patient's cancer types, sub-types, and/or stages, may also determine the patient's clinical response to therapies.

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## VARIANT INTERPRETATION

### *CHEK2* Heterozygous deletion

#### Biological Impact

The checkpoint kinase 2 (CHEK2 or CHK2) gene encodes a serine/threonine protein kinase involved in transducing DNA damage signals that are required for both the intra-S phase and G2/M checkpoints[1]. CHEK2 heterozygosity has been shown to cause haploinsufficient phenotypes that can contribute to tumorigenesis through inappropriate S phase entry, accumulation of DNA damage during replication, and failure to restrain mitotic entry[2][3]. CHEK2 aberrations are associated with glioblastoma, breast, ovarian, prostate, colorectal, gastric, thyroid, and lung cancers[4][5][6][7][8].

#### Therapeutic and prognostic relevance

Olaparib is FDA-approved for treating mCRPC patients harboring mutations in homologous recombination repair (HRR) genes, including CHEK2.

CHEK2 mutation has been determined as an inclusion criterion for the trials evaluating olaparib, rucaparib, niraparib, and talazoparib efficacies in various types of solid tumors (NCT03297606, NCT01968213, NCT03840967, NCT02401347, NCT03148795).

In TBCRC 048 trial, olaparib treatment did not show response in 7 metastatic breast cancer patients with germline mutations in CHEK2 (SD: n=3, PD: n=4)[9]. In TRITON2 trial, rucaparib treatment had limited response in 12 mCRPC patients with CHEK2 alterations[10].

### *NF2* Heterozygous deletion

#### Biological Impact

The neurofibromin (NF2) gene encodes the protein Merlin, a tumor suppressor that functions as a negative regulator of the PI3K/AKT/mTOR pathway[11][12][13]. NF2 is a haploinsufficient tumor suppressor gene with one copy loss may lead to weak protein expression and is insufficient to execute its original physiological functions[14]. Inactivation germline mutations in the NF2 are associated with the hereditary neurofibromatosis type 2, a disorder characterized by the growth of noncancerous tumors in the nervous system[11][15]. Somatic mutations or deletion of NF2 are frequently observed in human cancers, including 20-50% of pleural mesotheliomas[16], 6% papillary renal cell carcinoma, 5% pancreas cancer, and 4% melanoma (cbioPortal; June 2015), and less frequently in other cancers[17].

#### Therapeutic and prognostic relevance

Genomic alterations with activating effects on the mTOR signaling pathway have been identified to confer sensitivity to everolimus across multiple cancer types[18][19][20][21]. There are at least two case studies indicating the clinical efficacy of everolimus in bladder cancer and urothelial carcinoma[22][23], both harboring NF2 truncating mutations. Preclinical evidence has shown the efficacy of MEK1/2 inhibitor selumetinib in KRAS-mutant thyroid cancer model with NF2 loss[24].

Analysis of afatinib-plus-cetuximab-resistant biopsy specimens revealed a loss-of-function alteration in genes that modulate mTOR signaling pathway, including NF2 and TSC1[25].

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# US FDA-APPROVED DRUG(S)

## Everolimus (AFINITOR)

Everolimus, a derivative of sirolimus, works as an inhibitor of mammalian target of rapamycin complex 1 (mTORC1) and blocks mTORC1-mediated downstream signals for cell growth, proliferation, and survival. Everolimus is developed and marketed by Novartis under the trade name AFINITOR.

### - FDA Approval Summary of Everolimus (AFINITOR)

| | |
|---|---|
| **RADIANT-4**[26]<br>NCT01524783 | **Lung or gastrointestinal neuroendocrine tumor** (Approved on 2016/02/26) |
| | - |
| | Everolimus vs. Placebo [PFS(M): 11 vs. 3.9] |
| **BOLERO-2**[27]<br>NCT00863655 | **Breast cancer** (Approved on 2012/07/20) |
| | **ER+/HER2-** |
| | Everolimus + exemestane vs. Placebo + exemestane [PFS(M): 7.8 vs. 3.2] |
| **EXIST-2**<br>NCT00790400 | **Tuberous sclerosis complex (tsc)-associated renal angiomyolipoma** (Approved on 2012/04/26) |
| | - |
| | Everolimus vs. Placebo [ORR(%): 41.8 vs. 0] |
| **RADIANT-3**[28]<br>NCT00510068 | **Pancreatic neuroendocrine tumor** (Approved on 2011/05/05) |
| | - |
| | Everolimus vs. Placebo [PFS(M): 11 vs. 4.6] |
| **EXIST-1**[29]<br>NCT00789828 | **Subependymal giant cell astrocytoma** (Approved on 2010/10/29) |
| | - |
| | Everolimus vs. Placebo [ORR(%): 35.0] |
| **RECORD-1**[30]<br>NCT00410124 | **Renal cell carcinoma** (Approved on 2009/05/30) |
| | - |
| | Everolimus vs. Placebo [PFS(M): 4.9 vs. 1.9] |

## Niraparib (ZEJULA)

Niraparib is an oral, small molecule inhibitor of the DNA repair enzyme poly (ADP-ribose) polymerase-1 and -2 (PARP-1, -2). Niraparib is developed and marketed by Tesaro under the trade name ZEJULA.

### - FDA Approval Summary of Niraparib (ZEJULA)

| | |
|---|---|
| **PRIMA**<br>NCT02655016 | **Ovarian cancer, Fallopian tube cancer, Peritoneal carcinoma** (Approved on 2020/04/29) |
| | - |
| | Niraparib vs. Placebo [PFS (overall population)(M): 13.8 vs. 8.2] |
| **NOVA**[31]<br>NCT01847274 | **Ovarian cancer, Fallopian tube cancer, Peritoneal carcinoma** (Approved on 2017/03/27) |
| | - |
| | Niraparib vs. Placebo [PFS (overall population)(M): 11.3 vs. 4.7] |

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## Olaparib (LYNPARZA)

Olaparib is an oral, small molecule inhibitor of poly (ADP-ribose) polymerase-1, -2, and -3 (PARP-1, -2, -3). Olaparib is developed by KuDOS Pharmaceuticals and marketed by AstraZeneca under the trade name LYNPARZA.

## - FDA Approval Summary of Olaparib (LYNPARZA)

| | |
|---|---|
| **OlympiA**<br>NCT02032823 | **Her2-negative high-risk early breast cancer** (Approved on 2022/03/11) |
| | **HER2-/gBRCA mutation** |
| | Olaparib vs. Placebo [ invasive disease-free survival (IDFS)(M): ] |
| **PROfound**[32]<br>NCT02987543 | **Prostate cancer** (Approved on 2020/05/19) |
| | **HRR genes mutation** |
| | Olaparib vs. Enzalutamide or abiraterone acetate [PFS(M): 5.8 vs. 3.5] |
| **PAOLA-1**[33]<br>NCT02477644 | **Ovarian cancer** (Approved on 2020/05/08) |
| | **HRD+** |
| | Olaparib + bevacizumab vs. Placebo + bevacizumab [PFS(M): 37.2 vs. 17.7] |
| **POLO**[34]<br>NCT02184195 | **Pancreatic adenocarcinoma** (Approved on 2019/12/27) |
| | **gBRCA mutation** |
| | Olaparib vs. Placebo [ORR(%): 23.0 vs. 12.0, PFS(M): 7.4 vs. 3.8] |
| **SOLO-1**[35]<br>NCT01844986 | **Ovarian cancer, Fallopian tube cancer, Peritoneal carcinoma** (Approved on 2018/12/19) |
| | **gBRCA mutation or sBRCA mutation** |
| | Olaparib vs. Placebo [PFS(M): NR vs. 13.8] |
| **OlympiAD**[36]<br>NCT02000622 | **Breast cancer** (Approved on 2018/02/06) |
| | **HER2-/gBRCA mutation** |
| | Olaparib vs. Chemotherapy [PFS(M): 7 vs. 4.2] |
| **SOLO-2/ENGOT-Ov21**[37]<br>NCT01874353 | **Ovarian cancer, Fallopian tube cancer, Peritoneal carcinoma** (Approved on 2017/08/17) |
| | **gBRCA mutation** |
| | Olaparib vs. Placebo [PFS(M): 19.1 vs. 5.5] |
| **Study19**[38]<br>NCT00753545 | **Ovarian cancer, Fallopian tube cancer, Peritoneal carcinoma** (Approved on 2017/08/17) |
| | **-** |
| | Olaparib vs. Placebo [PFS(M): 8.4 vs. 4.8] |

## Rucaparib (RUBRACA)

Rucaparib is an inhibitor of the DNA repair enzyme poly (ADP-ribose) polymerase-1, -2 and -3 (PARP-1, -2, -3). Rucaparib is developed and marketed by Clovis Oncology under the trade name RUBRACA.

## - FDA Approval Summary of Rucaparib (RUBRACA)

| | |
|---|---|
| **TRITON2**<br>NCT02952534 | **Prostate cancer** (Approved on 2020/05/15) |
| | **gBRCA mutation or sBRCA mutation** |
| | Rucaparib [ORR(%): 44.0, DOR(M): NE] |
| **ARIEL3**[39]<br>NCT01968213 | **Ovarian cancer, Fallopian tube cancer, Peritoneal carcinoma** (Approved on 2018/04/06) |
| | **-** |
| | Rucaparib vs. Placebo [PFS (All)(M): 10.8 vs. 5.4, PFS (HRD)(M): 13.6 vs. 5.4, PFS (tBRCA)(M): 16.6 vs. 5.4] |

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## Talazoparib (TALZENNA)

Talazoparib is an inhibitor of poly (ADP-ribose) polymerase (PARP) enzymes, including PARP1 and PARP2. Talazoparib is developed and marketed by Pfizer under the trade name TALZENNA.

## - FDA Approval Summary of Talazoparib (TALZENNA)

| | |
|---|---|
| **EMBRACA**[40] NCT01945775 | **Breast cancer** (Approved on 2018/10/16) |
| | **HER2-/gBRCA mutation** |
| | Talazoparib vs. Chemotherapy [PFS(M): 8.6 vs. 5.6] |

## Temsirolimus (TORISEL)

Temsirolimus is a soluble ester of sirolimus (rapamycin, brand-name drug Rapamune) and functions as an inhibitor of mammalian target of rapamycin complex (mTORC). The inhibitory molecular mechanism is similar to Everolimus. Temsirolimus is developed by Wyeth Pharmaceuticals and marketed by Pfizer under the trade name TORISEL.

## - FDA Approval Summary of Temsirolimus (TORISEL)

| | |
|---|---|
| [41] NCT00065468 | **Renal cell carcinoma** (Approved on 2007/05/30) |
| | - |
| | Temsirolimus vs. Ifn-α [OS(M): 10.9 vs. 7.3] |

D=day; W=week; M=month

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## ONGOING CLINICAL TRIALS

Trials were searched by applying filters: study status, patient's diagnosis, intervention, location and/or biomarker(s). Please visit https://clinicaltrials.gov to search and view for a complete list of open available and updated matched trials.

No trial has been found.

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## SUPPLEMENTARY INFORMATION OF TESTING RESULTS

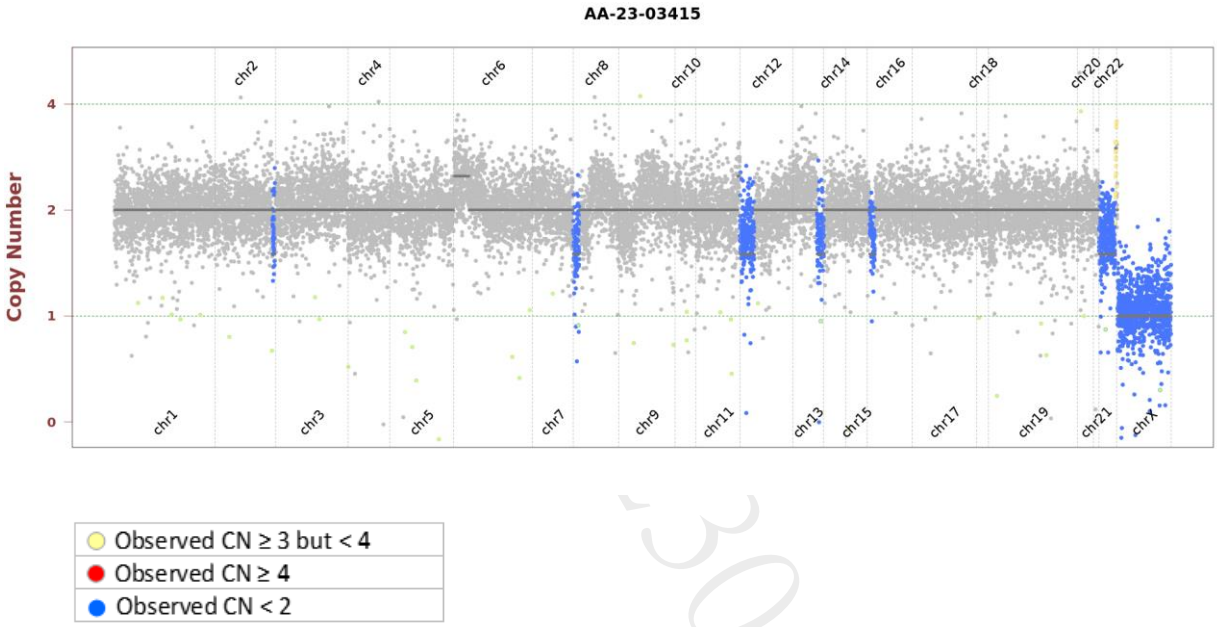
### DETAILED INFORMATION OF VARIANTS WITH CLINICAL RELEVANCE

#### - Single Nucleotide and Small InDel Variants

| Gene | Amino Acid Change | Exon | cDNA Change | Accession Number | COSMIC ID | Allele Frequency | Coverage |
|---|---|---|---|---|---|---|---|
| Not Detected | | | | | | | |

#### - Copy Number Alterations

Observed copy number (CN) for each evaluated position is shown on the y-axis. Regions referred to as amplification or deletion are shown in color. Regions without significant changes are represented in gray.



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## OTHER DETECTED VARIANTS

| Gene | Amino Acid Change | Exon | cDNA Change | Accession Number | COSMIC ID | Allele Frequency | Coverage |
|---|---|---|---|---|---|---|---|
| ADAMTS9 | P930H | 19 | c.2789C>A | NM_182920 | - | 50.2% | 1339 |
| FAT1 | E1292K | 5 | c.3874G>A | NM_005245 | - | 53.3% | 795 |
| KMT2D | Q3910_Q3911del | 39 | c.11729_11734del | NM_003482 | COSM249222 | 37.2% | 1337 |
| MED12 | I2050V | 42 | c.6148A>G | NM_005120 | COSM9264786 | 100.0% | 208 |
| MUC4 | P23L | 1 | c.68C>T | NM_018406 | - | 54.3% | 1655 |
| MYC | Splice region | - | c.30+8C>G | NM_002467 | - | 48.4% | 525 |
| NEFH | S787R | 4 | c.2361C>G | NM_021076 | - | 39.5% | 547 |
| PSME3 | Splice region | - | c.293-3C>T | NM_176863 | - | 44.0% | 1038 |
| RAD51D | R239W | 8 | c.715C>T | NM_002878 | - | 48.1% | 1059 |
| RB1 | R621S | 19 | c.1861C>A | NM_000321 | COSM1046 | 50.1% | 775 |
| SYNE1 | T2853A | 55 | c.8557A>G | NM_182961 | - | 67.4% | 1171 |
| USH2A | T2310M | 36 | c.6929C>T | NM_206933 | - | 51.0% | 1263 |
| USH2A | V1296A | 18 | c.3887T>C | NM_206933 | COSM1270032 | 57.7% | 376 |

Note:

- This table enlists variants detected by the panel other than those with clinical relevance (reported in Testing Result section). The clinical impact of a genetic variant is determined according to ACT Genomics in-house clinical knowledge database. A negative result does not necessarily indicate absence of biological effect on the tumor. Some variants listed here may possibly have preclinical data or may show potential clinical relevance in the future.

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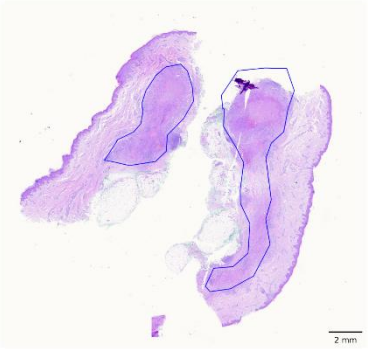
## TEST DETAILS

### SPECIMEN RECEIVED AND PATHOLOGY REVIEW




- Collection date: Apr 25, 2023
- Facility retrieved: 臺北榮總
- H&E-stained section No.: S11218466A
- Collection site: Soft tissue, forearm
- Examined by: Dr. Yun-An Chen
  1. The percentage of viable tumor cells in total cells in the whole slide (%): 20%
  2. The percentage of viable tumor cells in total cells in the encircled areas in the whole slide (%): 55%
  3. The percentage of necrotic cells (including necrotic tumor cells) in total cells in the whole slide (%): 10%
  4. The percentage of necrotic cells (including necrotic tumor cells) in total cells in the encircled areas in the whole slide (%): 15%
  5. Additional comment: NA
- Manual macrodissection: Performed on the highlighted region
- The outline highlights the area of malignant neoplasm annotated by a pathologist.

### RUN QC

- Panel: ACTOnco®+

**DNA test**

- Mean Depth: 909x
- Target Base Coverage at 100x: 95%

**RNA test**

- Average unique RNA Start Sites per control GSP2: 112

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## LIMITATIONS

1. This test does not provide information of variant causality and does not detect variants in non-coding regions that could affect gene expression. This report does not report polymorphisms and we do not classify whether a mutation is germline or somatic. Variants identified by this assay were not subject to validation by Sanger or other technologies.
2. The possibility cannot be excluded that certain pathogenic variants detected by other sequencing tools may not be reported in the test because of technical limitation of bioinformatics algorithm or the NGS sequencing platform, e.g. low coverage.
3. This test has been designed to detect fusions in 13 genes sequenced. Therefore, fusion in genes not covered by this test would not be reported. For novel fusions detected in this test, Sanger sequencing confirmation is recommended if residue specimen is available.

## NEXT-GENERATION SEQUENCING (NGS) METHODS

**DNA test**

Extracted genomic DNA was amplified using primers targeting coding exons of analyzed genes and subjected to library construction. Barcoded libraries were subsequently conjugated with sequencing beads by emulsion PCR and enriched using Ion Chef system. Sequencing was performed according to Ion Proton or Ion S5 sequencer protocol (Thermo Fisher Scientific).

Raw reads generated by the sequencer were mapped to the hg19 reference genome using the Ion Torrent Suite. Coverage depth was calculated using Torrent Coverage Analysis plug-in. Single nucleotide variants (SNVs) and short insertions/deletions (InDels) were identified using the Torrent Variant Caller plug-in. VEP (Variant Effect Predictor) was used to annotate every variant using databases from Clinvar, COSMIC and Genome Aggregation database. Variants with coverage ≥ 20, allele frequency ≥ 5% and actionable variants with allele frequency ≥ 2% were retained. This test provides uniform coverage of the targeted regions, enabling target base coverage at 100x ≥ 85% with a mean coverage ≥ 500x.

Variants reported in Genome Aggregation database with > 1% minor allele frequency (MAF) were considered as polymorphisms. ACT Genomics in-house database was used to determine technical errors. Clinically actionable and biologically significant variants were determined based on the published medical literature.

The copy number alterations (CNAs) were predicted as described below:
Amplicons with read counts in the lowest 5th percentile of all detectable amplicons and amplicons with a coefficient of variation ≥ 0.3 were removed. The remaining amplicons were normalized to correct the pool design bias. ONCOCNV (an established method for calculating copy number aberrations in amplicon sequencing data by Boeva et al., 2014) was applied for the normalization of total amplicon number, amplicon GC content, amplicon length, and technology-related biases, followed by segmenting the sample with a gene-aware model. The method was used as well for establishing the baseline of copy number variations.

Tumor mutational burden (TMB) was calculated by using the sequenced regions of ACTOnco®+ to estimate the number of somatic nonsynonymous mutations per megabase of all protein-coding genes (whole exome). The TMB calculation predicted somatic variants and applied a machine learning model with a cancer hotspot correction. TMB may be reported as “TMB-High”, “TMB-Low” or “Cannot Be Determined”. TMB-High corresponds to ≥ 7.5 mutations per megabase (Muts/Mb); TMB-Low corresponds to < 7.5 Muts/Mb. TMB is reported as “Cannot Be Determined” if the tumor purity of the sample is < 30%.

Classification of microsatellite instability (MSI) status is determined by a machine learning prediction algorithm. The change of a number of repeats of different lengths from a pooled microsatellite stable (MSS) baseline in > 400 genomic loci are used as the features for the algorithm. The final output of the results is either microsatellite Stable (MSS) or microsatellite instability high (MSI-H).

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**RNA test**

Extracted RNA was reverse-transcribed and subjected to library construction. Sequencing was performed according to Ion Proton or Ion S5 sequencer protocol (Thermo Fisher Scientific). To ensure sequencing quality for fusion variant analysis, the average unique RNA Start Sites (SS) per control Gene Specific Primer 2 (GSP 2) should be ≥ 10.

The fusion analysis pipeline aligned sequenced reads to the human reference genome, identified regions that map to noncontiguous regions of the genome, applied filters to exclude probable false-positive events and, annotated previously characterized fusion events according to Quiver Gene Fusion Database, a curated database owned and maintained by ArcherDX. In general, samples with detectable fusions need to meet the following criteria: (1) Number of unique start sites (SS) for the GSP2 ≥ 3; (2) Number of supporting reads spanning the fusion junction ≥ 5; (3) Percentage of supporting reads spanning the fusion junction ≥ 10%; (4) Fusions annotated in Quiver Gene Fusion Database.

## DATABASE USED

- Reference genome: Human genome sequence hg19
- COSMIC v.92
- Genome Aggregation database r2.1.1
- ClinVar (version 20210404)
- ACT Genomics in-house database
- Quiver Gene Fusion Database version 5.1.18

**Variant Analysis:**
醫檢師黃靖婷 博士
Ching-Ting Huang Ph.D.
檢字第 016511 號

CT Huang

**Sign Off**
解剖病理專科醫師王業翰
Yeh-Han Wang M.D.
病解字第 000545 號

Yeh

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## GENE LIST

### SNV & CNV

| | | | | | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|---|---|
| ABCB1* | ABCC2* | ABCG2* | ABL1 | ABL2 | ADAMTS1 | ADAMTS13 | ADAMTS15 | ADAMTS16 | ADAMTS18 | ADAMTS6 | ADAMTS9 |
| ADAMTSL1 | ADGRA2 | ADH1C* | AKT1 | AKT2 | AKT3 | ALDH1A1* | ALK | AMER1 | APC | AR | ARAF |
| ARID1A | ARID1B | ARID2 | ASXL1 | ATM | ATR | ATRX | AURKA | AURKB | AXIN1 | AXIN2 | AXL |
| B2M | BAP1 | BARD1 | BCL10 | BCL2* | BCL2L1 | BCL2L2* | BCL6 | BCL9 | BCOR | BIRC2 | BIRC3 |
| BLM | BMPR1A | BRAF | BRCA1 | BRCA2 | BRD4 | BRIP1 | BTG1 | BTG2* | BTK | BUB1B | CALR |
| CANX | CARD11 | CASP8 | CBFB | CBL | CCNA1 | CCNA | CCNB1 | CCNB2 | CCNB3 | CCND1 | CCND2 |
| CCND3 | CCNE1 | CCNE2 | CCNH | CD19 | CD274 | CD58 | CD70* | CD79A | CD79B | CDC73 | CDH1 |
| CDK1 | CDK12 | CDK2 | CDK4 | CDK5 | CDK6 | CDK7 | CDK8 | CDK9 | CDKN1A | CDKN1B | CDKN2A |
| CDKN2B | CDKN2C | CEBPA* | CHEK1 | CHEK2 | CIC | CREBBP | CRKL | CRLF2 | CSF1R | CTCF | CTLA4 |
| CTNNA1 | CTNNB1 | CUL3 | CYLD | CYP1A1* | CYP2B6* | CYP2C19* | CYP2C8* | CYP2D6 | CYP2E1* | CYP3A4* | CYP3A5* |
| DAXX | DCUN1D1 | DDR2 | DICER1 | DNMT3A | DOT1L | DPYD | DTX1 | E2F3 | EGFR | EP300 | EPCAM |
| EPHA2 | EPHA3 | EPHA5 | EPHA7 | EPHB1 | ERBB2 | ERBB3 | ERBB4 | ERCC1 | ERCC2 | ERCC3 | ERCC4 |
| ERCC5 | ERG | ESR1 | ESR2 | ETV1 | ETV4 | EZH2 | FAM46C | FANCA | FANCC | FANCD2 | FANCE |
| FANCF | FANCG | FANCL | FAS | FAT1 | FBXW7 | FCGR2B | FGF1* | FGF10 | FGF14 | FGF19* | FGF23 |
| FGF3 | FGF4* | FGF6 | FGFR1 | FGFR2 | FGFR3 | FGFR4 | FH | FLCN | FLT1 | FLT3 | FLT4 |
| FOXL2* | FOXP1 | FRG1 | FUBP1 | GATA1 | GATA2 | GATA3 | GNA11 | GNA13 | GNAQ | GNAS | GREM1 |
| GRIN2A | GSK3B | GSTP1* | GSTT1* | HGF | HIF1A | HIST1H1C* | HIST1H1E* | HNF1A | HR | HRAS* | HSP90AA1 |
| HSP90AB1 | HSPA4 | HSPA5 | IDH1 | IDH2 | IFNL3* | IGF1 | IGF1R | IGF2 | IKBKB | IKBKE | IKZF1 |
| IL6 | IL7R | INPP4B | INSR | IRF4 | IRS1 | IRS2* | JAK1 | JAK2 | JAK3 | JUN* | KAT6A |
| KDM5A | KDM5C | KDM6A | KDR | KEAP1 | KIT | KMT2A | KMT2C | KMT2D | KRAS | LCK | LIG1 |
| LIG3 | LMO1 | LRP1B | LYN | MALT1 | MAP2K1 | MAP2K2 | MAP2K4 | MAP3K1 | MAP3K7 | MAPK1 | MAPK3 |
| MAX | MCL1 | MDM2 | MDM4 | MED12 | MEF2B | MEN1 | MET | MITF | MLH1 | MPL | MRE11 |
| MSH2 | MSH6 | MTHFR* | MTOR | MUC16 | MUC4 | MUC6 | MUTYH | MYC | MYCL | MYCN | MYD88 |
| NAT2* | NBN | NEFH | NF1 | NF2 | NFE2L2 | NFKB1 | NFKBIA | NKX2-1* | NOTCH1 | NOTCH2 | NOTCH3 |
| NOTCH4 | NPM1 | NQO1* | NRAS | NSD1 | NTRK1 | NTRK2 | NTRK3 | PAK3 | PALB2 | PARP1 | PAX5 |
| PAX8 | PBRM1 | PDCD1 | PDCD1LG2 | PDGFRA | PDGFRB | PDIA3 | PGF | PHOX2B* | PIK3C2B | PIK3C2G | PIK3C3 |
| PIK3CA | PIK3CB | PIK3CD | PIK3CG | PIK3R1 | PIK3R2 | PIK3R3 | PIM1 | PMS1 | PMS2 | POLB | POLD1 |
| POLE | PPARG | PPP2R1A | PRDM1 | PRKAR1A | PRKCA | PRKCB | PRKCG | PRKCI | PRKCQ | PRKDC | PRKN |
| PSMB8 | PSMB9 | PSME1 | PSME2 | PSME3 | PTCH1 | PTEN | PTGS2 | PTPN11 | PTPRD | PTPRT | RAC1 |
| RAD50 | RAD51 | RAD51B | RAD51C | RAD51D | RAD52 | RAD54L | RAF1 | RARA | RB1 | RBM10 | RECQL4 |
| REL | RET | RHOA | RICTOR | RNF43 | ROS1 | RPPH1 | RPTOR | RUNX1 | RUNX1T1 | RXRA | SDHA |
| SDHB | SDHC | SDHD | SERPINB3 | SERPINB4 | SETD2 | SF3B1 | SGK1 | SH2D1A* | SLC19A1* | SLC22A2* | SLCO1B1* |
| SLCO1B3* | SMAD2 | SMAD3 | SMAD4 | SMARCA4 | SMARCB1 | SMO | SOCS1* | SOX2* | SOX9 | SPEN | SPOP |
| SRC | STAG2 | STAT3 | STK11 | SUFU | SYK | SYNE1 | TAF1 | TAP1 | TAP2 | TAPBP | TBX3 |
| TEK | TERT | TET1 | TET2 | TGFBR2 | TMSB4X* | TNF | TNFAIP3 | TNFRSF14 | TNFSF11 | TOP1 | TP53 |
| TPMT* | TSC1 | TSC2 | TSHR | TYMS | U2AF1 | UBE2A* | UBE2K | UBR5 | UGT1A1* | USH2A | VDR* |
| VEGFA | VEGFB | VHL | WT1 | XIAP | XPO1 | XRCC2 | ZNF217 | | | | |

*Analysis of copy number alterations NOT available.

### FUSION

| | | | | | | | | | | | | |
|---|---|---|---|---|---|---|---|---|---|---|---|---|
| ALK | BRAF | EGFR | FGFR1 | FGFR2 | FGFR3 | MET | NRG1 | NTRK1 | NTRK2 | NTRK3 | RET | ROS1 |

行動基因僅提供技術檢測服務及檢測報告，檢測結果之臨床解釋及相關醫療處置，請諮詢專業醫師。報告結果僅對此試驗件有效。
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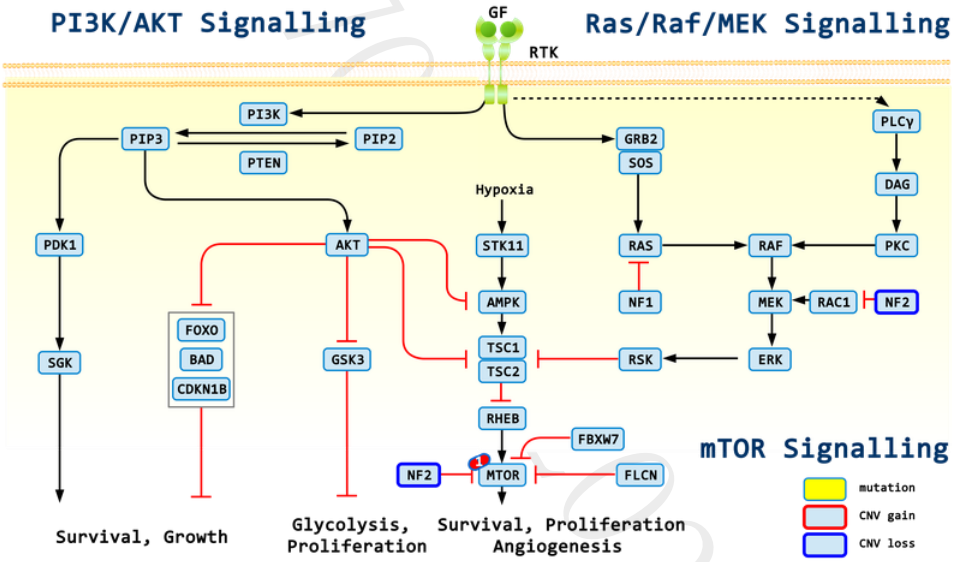
# APPENDIX

## POSSIBLE THERAPEUTIC IMPLICATIONS FOR HETEROZYGOUS DELETION

| Gene | Therapies | Possible effect |
|---|---|---|
| *NF2* | Everolimus, Temsirolimus | **sensitive** |
| *CHEK2* | Niraparib, Olaparib, Rucaparib, Talazoparib | **sensitive** |

## SIGNALING PATHWAYS AND MOLECULAR-TARGETED AGENTS




**1: Everolimus, Temsirolimus**

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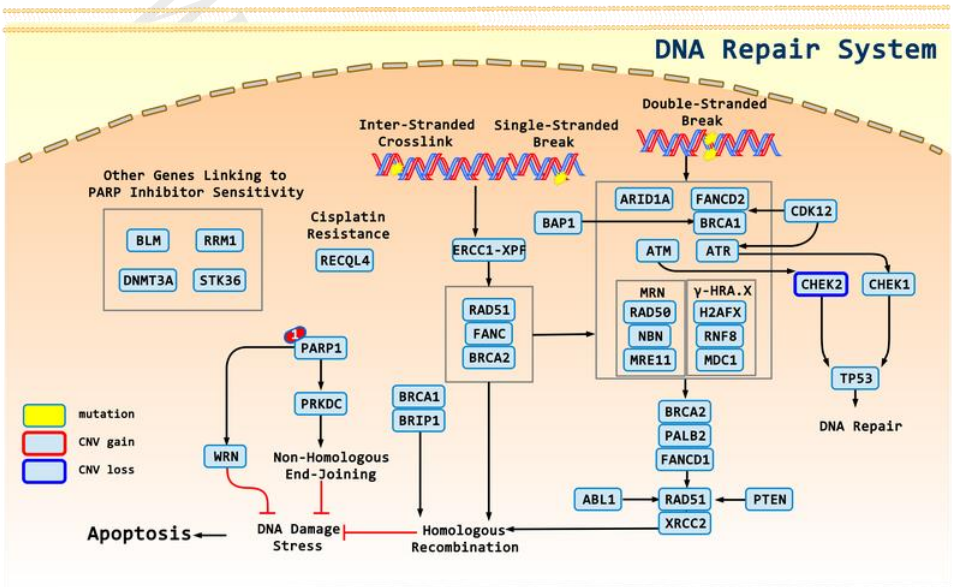


**1: Olaparib, Niraparib, Rucaparib, Talazoparib**

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## DISCLAIMER

### 法律聲明

本檢驗報告僅提供專業醫療參考，結果需經專業醫師解釋及判讀。基因突變資訊非必具備藥物或治療有效性指標，反之亦然。本檢驗報告提供之用藥指引不聲明或保證其臨床有效性，反之亦然。本基因檢測方法係由本公司研究開發，已經過有效性測試。
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### 醫療決策需由醫師決定

任何治療與用藥需經由醫師在考慮病患所有健康狀況相關資訊包含健檢、其他檢測報告和病患意願後，依照該地區醫療照護標準由醫師獨立判斷。醫師不應僅依據單一報告結果(例如本檢測或本報告書內容)做決策。

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藥物潛在臨床效益(或缺乏潛在臨床效益)的實證證據是依據至少一篇臨床療效個案報告或臨床前試驗做為評估。本公司盡力提供適時及準確之資料，但由於醫學科技之發展日新月異，本公司不就本報告提供的資料是否為準確、適宜或最新作保證。

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